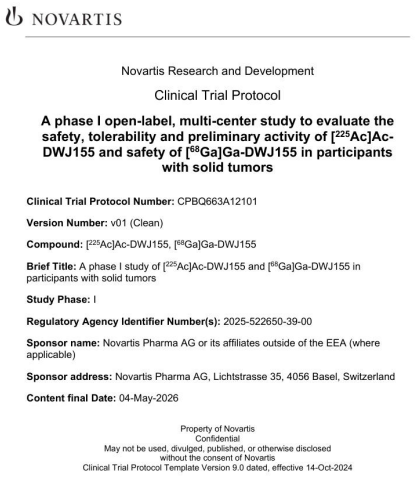
DWJ155

CPBQ663A12101 · ^{68}Ga / ^{225}Ac DWJ155 · Phase I theranostic

Novartis · P000000 in Board Studio



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Clinical Trial Protocol

A phase I open-label, multi-center study to evaluate the safety, tolerability and preliminary activity of [^{225}Ac]Ac-DWJ155 and safety of [^{68}Ga]Ga-DWJ155 in participants with solid tumors

Clinical Trial Protocol Number: CPBQ663A12101

Version Number: v01 (Clean)

Compound: [^{225}Ac]Ac-DWJ155, [^{68}Ga]Ga-DWJ155

Brief Title: A phase I study of [^{225}Ac]Ac-DWJ155 and [^{68}Ga]Ga-DWJ155 in participants with solid tumors

Study Phase: I

Regulatory Agency Identifier Number(s): 2025-522650-39-00

Sponsor name: Novartis Pharma AG or its affiliates outside of the EEA (where applicable)

Sponsor address: Novartis Pharma AG, Lichtstrasse 35, 4056 Basel, Switzerland

Content final Date: 04-May-2026

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Clinical Trial Protocol Template Version 9.0 dated, effective 14-Oct-2024

What this study is, in one breath

Same HER2-targeting peptide: gallium-68 PET to see the target, actinium-225 alpha to treat it, in HER2-spectrum solid tumors.

Not trastuzumab. Not T-DXd. Imaging gates therapy.

> **If the protocol feels like a wall of words**
>
> A protocol is a recipe: who can join, what is injected, how much, when you scan or treat, and how you decide if a picture or outcome was right. This sheet is the same idea in normal English. It does not replace the signed site copy.

1. Theranostic pair

Novartis CPBQ663A12101 is first-in-human Phase I of DWJ155, a HER2-targeting peptide. Same ligand, two radionuclides: ^{68}Ga for PET (see the target) and ^{225}Ac for alpha therapy (treat the target). Not trastuzumab, not T-DXd, not FDG.

IMP	Role	Starting numbers
^{68}Ga-DWJ155	Imaging · PET/CT or PET/MRI	~250 MBq (window 200–275). Screening + end of treatment.
^{225}Ac-DWJ155	Therapy · α emitter	Start 2 MBq (54 µCi) Q6W, up to 4 cycles. Provisional 2 / 4 / 6 / 8 MBq.

Positive imaging PET: at least one measurable lesion with DWJ155 uptake. That scan gates whether you get actinium. Mass dose ~40 µg at start, up to ~400 µg if activity climbs.

2. Who

Solid tumors across the HER2 spectrum (IHC 3+ down to IHC 0 — the PET is the in-vivo IHC): breast, NSCLC ± AGA, gastroesophageal, bladder. Escalation ~21–48 people; expansion ~135; 36-month follow-up.

3. Escalation rules

- BHLRM (Bayesian hierarchical logistic regression) plus EWOC: do not escalate into an excessive-toxicity posterior.
- Q6W opens first; Q8W and Q4W can follow under the same model.
- DLT window = first cycle (4, 6, or 8 weeks matching the schedule).
- Recommended dose = activity per cycle + number of cycles + schedule, not a single number.

> **Alpha vs PET**
>
> Do not swap the names. Gallium maps. Actinium treats. Organs at risk are radiation to normal tissue, not cold-peptide pharmacology.

What to know cold

Q. Pair?
A. ^{68}Ga-DWJ155 PET (~250 MBq) and ^{225}Ac-DWJ155 therapy (start 2 MBq Q6W).

Q. Who?
A. Breast, NSCLC, GEA, bladder across HER2 3+ down to IHC 0 if PET is positive.

Q. Escalation?
A. BHLRM + EWOC. Provisional 2 / 4 / 6 / 8 MBq. Q6W first.

Q. Gate?
A. At least one measurable lesion with DWJ155 uptake on the screening PET.

Q. RD means?
A. Activity per cycle + number of cycles + schedule.

How to use this with Board Studio

Open the matching protocol module and press play. The left pane is the signed page. Use this PDF when legalese stalls you. If anything conflicts, the signed protocol wins.

Disclaimer. Educational summary for board-style study. Not medical advice, not a recruitment brochure, not a substitute for the signed protocol, and not affiliated with the sponsor, ABR, ABNM, ACGME, SNMMI, NRC, or FDA. Confirm numbers against the current site copy before any clinical use.